

Patient Demographics

Patient Demographics				
Name	Patient ID	SSN	Gender Identity	Birth Date
Stefonowicz, Todd Michael	21997342	xxx-xx-4580	Male	07/10/62 (64 yrs)
Address	Phone	Email		
1751 FAIR WAY CARSON CITY NV 89701	775-544-7485 (M)	tstefono@charter.net		
Reg Status	PCP	Date Last Verified	Next Review Date	
Verified Registration	Jeffrey Woodrow Sanders, MD775-882-1324	05/13/26	11/09/26	

Stefonowicz, Todd Michael #21997342
(Acct:8209348359) (64 year old M) PCP: SANDERS, J
(775-882-1324)

01

ED Arrival Information

ED Arrival Information			
Expected	Arrival	Acuity	
-	7/10/2026 18:53	Level 3	
Means of arrival	Escorted by	Service	Admission type
Private Transportation	Spouse	General Medicine	Emergency

Arrival complaint
rectal bleeding, 8 days s/p colonoscopy

Chief Complaint

Complaint	Comment
Blood In Stool Rectal Bleeding	

ED Treatment Team

Provider	Role	From	To	Phone	Pager
Jeremy Mark Gonda, MD	Attending Provider	07/10/26 1940	07/10/26 2130	775-445-8712	--
Isabella Griffin, RN	Registered Nurse	07/10/26 1929	07/10/26 2236	--	--
Ryan Monti, MD	Admitting Provider	--	--	775-445-8000	--

ED Provider Notes**ED Provider Notes by Jeremy Mark Gonda, MD**

07/10/26 1940

Author: Jeremy Mark Gonda, MD Service: Emergency Author Type: Physician
Filed: 07/10/26 2134 Date of Service: 07/10/26 1940 Status: Signed
Editor: Jeremy Mark Gonda, MD (Physician)

EMERGENCY MEDICINE PHYSICIAN NOTE**DATE OF SERVICE**

07/10/26

MODE OF ARRIVALMeans of Arrival:^[JG.1] Private Transportation^[JG.2]

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

CHIEF COMPLAINT^[JG.1]

Chief Complaint

Patient presents with

- Blood In Stool
- Rectal Bleeding^[JG.2]

HISTORY OF PRESENT ILLNESS (HPI)Patient ID: Todd Michael Stefonowicz is a 64 year old male with chief complaint of^[JG.1]

Chief Complaint

Patient presents with

- Blood In Stool
- Rectal Bleeding^[JG.2]

PCP, if available:^[JG.1] Jeffrey W Sanders^[JG.2]

(History obtained from patient, bedside RN, review of past medical records, and patient's spouse.)

Patient is a pleasant 64-year-old male with a past medical history significant for erectile dysfunction who underwent his 1st routine screening colonoscopy after a positive Cologuard test on July 2nd with Dr. Reddy. He was found to have diverticulosis, a few polyps including a large 4 x 5 cm polyp that was subsequently removed with clips placed. It came back with tubular adenoma on pathology. He was doing fine postprocedure until this evening when he had a sudden urge to have a bowel movement and had a large, bloody bowel movement. He had associated lightheadedness and dizziness but denied abdominal pain. He denies any nausea, vomiting, fevers, chills. He did eat spicy breakfast this morning and took some Tums and denies any history of upper gastrointestinal hemorrhage nor ulcer disease. He was able to contact Gastroenterology Dr. Reddy who recommended he come into the emergency department for further evaluation. He does not use any blood thinners nor over-the-counter supplements that could thin his blood.

REVIEW OF SYSTEMS (ROS)Review of Systems

Constitutional: Negative for chills, fatigue and fever.

HEENT: Negative for congestion and sore throat.

Eyes: Negative for discharge.

Respiratory: Negative for cough and shortness of breath.

Cardiovascular: Negative for chest pain and leg swelling.

Gastrointestinal: Positive for **blood in stool** and **anal bleeding**. Negative for nausea, vomiting, abdominal pain, diarrhea, abdominal distention and rectal pain.

Endocrine: Negative for polyuria.

Genitourinary: Negative for dysuria and urgency.

Musculoskeletal: Negative for back pain and myalgias.

Allergic/Immunologic: Negative for immunocompromised state.

Neurological: Positive for **dizziness** and **light-headedness**. Negative for numbness.

Hematological: Does not bruise/bleed easily.

Skin: Negative for rash.

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

Psychiatric/Behavioral: Negative for confusion. The patient is not nervous/anxious.
All other systems reviewed and are negative.

10 point review of systems negative except for HPI, or non-contributory to this visit.

PAST MEDICAL HISTORY^[JG.1]

History reviewed. No pertinent past medical history.^[JG.2]

PAST SURGICAL HISTORY^[JG.1]

Past Surgical History:

Procedure	Laterality	Date
• INGUINAL HERNIA REPAIR	Right	2022 ^[JG.2]

FAMILY HISTORY^[JG.1]

History reviewed. No pertinent family history.^[JG.2]

SOCIAL HISTORY

Social History

Social History

Tobacco Use

- Smoking status: Never
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Yes
 - Alcohol/week: 7.0 standard drinks of alcohol
 - Types: 7 Standard drinks or equivalent per week
- Drug use: Not Currently

Patient counseled on smoking cessation: N/ A - patient does not smoke

CURRENT MEDICATIONS

Current Medications

No current facility-administered medications for this encounter.

Current Outpatient Medications

Medication	Sig	Dispense	Refill
• psyllium (METAMUCIL) 58.6 % powder	Take by mouth once daily.	425 gram	3
• tadalafil (CIALIS) 20 mg tablet	1 tablet as needed Orally Once a day; Duration: 100 days ^[JG.1]		

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

ALLERGIES^[JG.1]No Known Allergies^[JG.2]*Nursing records including triage note, medical histories, medications, and allergies were also reviewed.***PHYSICAL EXAM** [press physical exam button]*I reviewed the vital signs including triage vitals***Vitals**^[JG.1]**Vitals**

	07/10/26 1925	07/10/26 2000	07/10/26 2052	07/10/26 2100
BP:	130/67	144/78	153/81	119/79
Pulse:	59	94	86	89
Temp:				
Resp:		20	20	20
Weight:				
SpO2:	96%	95%	98%	96%

BMI(Calculated)^[JG.2]

182.9 cm (6')

^[JG.1] 172 lbs 6.4 oz^[JG.2], ^[JG.1] Body mass index is 23.38 kg/m².^[JG.2]**Physical Exam**

Vitals and nursing note reviewed.

Constitutional:

General: He is awake. He is not in acute distress.

Appearance: He is well-developed and normal weight. He is not toxic-appearing.

HENT:

Head: Normocephalic and atraumatic.

Nose: Nose normal.

Mouth/ Throat:

Mouth: Mucous membranes are moist.

Pharynx: Oropharynx is clear.

Eyes:

General: No scleral icterus.

Conjunctiva/ sclera: Conjunctivae normal.

Pupils: Pupils are equal, round, and reactive to light.

Comments: No conjunctival pallor

Neck:

Vascular: No JVD.

Cardiovascular:Rate and Rhythm: Regular rhythm. **Bradycardia** present.

Pulses: Normal pulses.

Heart sounds: Normal heart sounds. No murmur heard.

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

Comments: Mildly hypertensive

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds. No rhonchi.

Chest:

Chest wall: No tenderness.

Abdominal:

General: Bowel sounds are normal. There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness. There is no right CVA tenderness, left CVA tenderness, guarding or rebound.

Genitourinary:Rectum: **Guaiac result positive.**

Comments: A few external and internal hernias without thrombosis nor tenderness, no anal fissure, stool is bright red

Musculoskeletal:

General: No tenderness. Normal range of motion.

Cervical back: Neck supple.

Skin:

General: Skin is warm and dry.

Capillary Refill: Capillary refill takes less than 2 seconds.

Coloration: Skin is not pale.

Findings: No bruising or erythema.

Neurological:

General: No focal deficit present.

Mental Status: He is alert and oriented to person, place, and time.

Cranial Nerves: No cranial nerve deficit.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal. Behavior is cooperative.

DIAGNOSTIC STUDIES:^[JG.1]

Recent Results (from the past 48 hours)

CBC With Plt Count And Auto Diff

Collection Time: 07/10/26 7:44 PM

Result	Value	Ref Range
WBC Count	8.8	3.7 - 10.6 x10E3/uL
RBC	5.07	4.50 - 5.70 x10e6/uL
Hemoglobin	16.0	13.0 - 16.7 g/dL
Hematocrit	45.8	38.8 - 49.7 %
Mean Corpuscular Volume	90.2	83.0 - 99.0 fL

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

Mean Corpuscular Hemoglobin	31.6	28.0 - 33.8 pg
Mean Corpuscular HGB Concentration	35.0	33.1 - 36.5 g/dL
Red Cell Distribution Width	12.8	11.8 - 14.0 %
Platelet Count	245	146 - 390 x10E3/uL
Mean Platelet Volume	8.1	6.4 - 10.2 fL
Neutrophil %	65.2	41.7 - 82.3 %
Lymphocyte %	24.2	10.8 - 44.4 %
Monocyte %	9.3	5.0 - 12.8 %
Eosinophil %	0.8	0.0 - 6.6 %
Basophil %	0.5	0.0 - 1.3 %
Neutrophil #	5.70	1.40 - 8.00 x10E3/uL
Lymphocyte #	2.10	1.00 - 5.20 x10E3/uL
Monocyte #	0.80	0.16 - 1.00 x10E3/uL
Eosinophil #	0.10	0.00 - 0.80 x10E3/uL
Basophil #	0.00	0.00 - 0.30 x10E3/uL

Comprehensive Metabolic Panel

Collection Time: 07/10/26 7:44 PM

Result	Value	Ref Range
Sodium, Serum or Plasma	134 (L)	136 - 144 mmol/L
Potassium	3.6	3.6 - 5.1 mmol/L
Chloride	102	102 - 110 mmol/L
Carbon Dioxide	24	22 - 32 mmol/L
Alkaline Phosphatase	57	38 - 126 U/L
AST	19	15 - 41 U/L
ALT	19	17 - 63 U/L
BUN	14	8 - 20 mg/dL
Creatinine, Serum or Plasma	1.19	0.80 - 1.40 mg/dL
Calcium	10.1	8.7 - 10.3 mg/dL
Protein, Total, Serum or Plasma	6.5	6.4 - 8.2 g/dL
Albumin	4.2	3.5 - 4.8 g/dL
A/G Ratio	1.8	1.0 - 2.2 ratio
Glomerular Filtration Rate (GFR)	62	>60 mL/min/1.7
Globulins, Total	2.3 (L)	2.6 - 3.1 g/dL
Glucose, Serum or Plasma	98	60 - 99 mg/dL
Bilirubin, Total	0.4	0.4 - 2.0 mg/dL
Anion Gap	8	2 - 11 mmol/L

Prothrombin Time/International Normalized Ratio

Collection Time: 07/10/26 7:44 PM

Result	Value	Ref Range
Prothrombin Time	14.2	11.7 - 14.4 seconds
INR	1.0	0.9 - 1.1 ratio

Creatinine (I-STAT)

Collection Time: 07/10/26 7:59 PM

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

Result	Value	Ref Range
Creatinine - POC	1.1	0.6 - 1.3 mg/dL

12 Lead ECG

Collection Time: 07/10/26 8:09 PM

Result	Value	Ref Range
Ventricular Rate	90	BPM
Atrial Rate	90	BPM
P-R Interval	150	ms
QRS Duration	96	ms
Q-T Interval	354	ms
QTC Calculation (Bezjet)	433	ms
Calculated P Axis	33	degrees
Calculated R Axis	-52	degrees
Calculated T Axis	31	degrees

Diagnosis

Normal sinus rhythm
Left anterior fascicular block
Abnormal ECG
No previous ECGs available

CT Abdomen Pelvis W IV Cont

Result Date: 7/10/2026

1. No definite acute abnormality in the abdomen or pelvis. No evidence of GI hemorrhage on significantly limited single contrast phase exam. Consider CT angiogram if clinical concerns persist. 2. Diverticulosis without diverticulitis. 3. Small hiatal hernia. 4. Diffuse hepatic steatosis. Electronically Signed by: Jonathan Hester 7/10/2026 9:06 PM^[JG.2]

CXR/CT/US/ Rhythm strip:

Personally reviewed and interpreted by me, ED physician to show: CT abdomen/pelvis without evidence of acute GI hemorrhage, diverticulosis without diverticulitis, fatty liver and small hiatal hernia

EKG:

07/10/26

Time: 2009

Personally reviewed and interpreted by me, ED physician to show: Normal sinus rhythm with a heart rate of 90 with a left anterior fascicular block, normal QTC and no evidence of acute ST-elevation

PROCEDURES:

Procedures - none

Nursing Review: FHx, PMHx, Social Hx, medications, allergies, and all applicable medical records/radiologic studies reviewed with nursing. This information may not have populated this note yet; however, this

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

information can be found in the nursing chart.

EMERGENCY DEPARTMENT COURSE/MEDICAL DECISION MAKING (MDM):**Differential Diagnosis:**

Based on the patient's HPI, risk factors, and physical examination the high risk differential diagnosis includes diverticulosis versus complication from recent colonoscopy/clip versus acute blood loss anemia versus thrombocytopenia versus coagulopathy versus malignancy. Doubt upper GI bleed.

SOCIAL DETERMINANTS OF HEALTH:

Additionally the patient's health is significantly affected by certain social determinants of health including: NA

[Medical records/external notes were reviewed for this visit including: I reviewed the colonoscopy report from July 2, 2026 including findings of large pedunculated polyp approximately 4-5 cm 45 cm from the anal verge requiring injection of epinephrine, resection with hot snare, Endoclip as well as severe diverticulosis, sessile polyp in the sigmoid colon and small internal hemorrhoids.

[Lab results Interpretation: CBC with differential within normal limits, CMP remarkable for a sodium low at 134 but otherwise within normal limits, INR 1.0

MEDICATIONS DURING ED COURSE^[JG.1]**Medications**

iopamidol (ISOVUE-370) 76 % injection 95 mL (95 mL Intravenous Given 7/10/26 2050)^[JG.2]

This patient required the following parenteral (intravenous) medications: N/A

The patient received the following medications which required intensive monitoring due to high risk of morbidity/mortality: N/A

[QHP discussions]

Discussed with Gastroenterology Dr. Takata @ 2114

Discuss with hospitalist Dr. Monti at 2130^[JG.1]

ED Course as of 07/10/26 2131

Fri Jul 10, 2026

2111 CT Abdomen Pelvis W IV Cont

I was contacted by the radiology tech who reports that he did not time the contrast for a CT angio as initially ordered. I reviewed the results which did not show evidence of active bleeding but does show diverticulosis. ^[JG]

2112 I have placed a call out to GI to discuss patient's symptoms and workup. ^[JG]

2114 I spoke with the covering gastroenterologist Dr. Takata who recommends patient be admitted to observation with serial hemoglobins, NPO at midnight and will plan on colonoscopy if he continues to bleed in the

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

morning. [JG]

2115 I spoke with the patient and his spouse who agree to the plan. I reached out to the hospitalist who agrees to the admission. [JG]

ED Course User Index

[JG] Jeremy Mark Gonda, MD

Clinical Impressions as of 07/10/26 2131

Lower GI bleed

Diverticulosis^[JG.2]**FINAL DIAGNOSIS:**^[JG.1]

Final diagnoses:

Lower GI bleed

Diverticulosis^[JG.2]

Primary Impression: Acute lower GI bleed after recent colonoscopy

Patient factors significantly impacting clinical decision making:

Relevant co-morbidities increasing baseline patient risk or complexity for the presenting complaint: Diverticular disease

Evaluations considered but deferred: Labs and Scans

Therapies considered but deferred: Discharge versus IV PPI versus octreotide versus IV fluids

Shared decision making Summary: Patient is a pleasant 64-year-old male who presents for the above concerns. He was placed on continuous telemetry and pulse oximetry monitoring and found to have normal vital signs. He's nontoxic appearing. His abdomen is soft and nontender. He underwent the above workup showing normal labs and a CT scan without obvious source of bleeding. I discussed with Gastroenterology and patient will be admitted for serial H/H, NPO at midnight, and possible repeat colonoscopy in the morning. Patient, spouse, hospitalists are agreeable to the plan. Patient is stable at time of discharge.

DISPOSITION:^[JG.1]Admit^[JG.2]

Admit physician: Dr. Monti

Request time: 2130

Request date: 07/10/26

Call information: Agrees with admission^[JG.1]

Final diagnoses:

Lower GI bleed

Diverticulosis

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

No follow-up provider specified.

No orders of the defined types were placed in this encounter.^[JG.2]

MEDICATIONS FOR HOME^[JG.1]**Patient's Medications****New Prescriptions**

No medications on file

Previous Medications

PSYLLIUM (METAMUCIL) Take by mouth once daily.
58.6 % POWDER

TADALAFIL (CIALIS) 20 MG 1 tablet as needed Orally
TABLET Once a day; Duration: 100
days

Modified Medications

No medications on file

Discontinued Medications

No medications on file^[JG.2]

Prescription management: N/A

Controlled Substances: N/A

Mandatory Screening for select patients: Sexually transmitted disease testing was offered in the emergency department: Not applicable. Syphilis screening was offered in the emergency department: not applicable as the patient was not pregnant.

Portions of this document may have been created by nursing or other staff, these were reviewed and accepted by me. Appropriate corrections were made. This document was created using a computerized dictation system. Although I have reviewed this document, there may still be some phonetic and other typographical errors due to imperfections in the software program and should not be construed in any way to compromise the patient's medical care.^[JG.1]

Jeremy M Gonda, MD^[JG.2]

Emergency Medicine Physician

Jeremy Mark Gonda, MD

07/10/26 2134

^[JG.3]

ED Provider Notes (continued)**ED Provider Notes by Jeremy Mark Gonda, MD (continued)**

07/10/26 1940

Attribution Key

JG.1 - Jeremy Mark Gonda, MD on 07/10/26 1940

JG.2 - Jeremy Mark Gonda, MD on 07/10/26 2131

JG.3 - Jeremy Mark Gonda, MD on 07/10/26 2134

ED Course

Value	Comment	By	Time
CT Abdomen Pelvis W IV Cont	I was contacted by the radiology tech who reports that he did not time the contrast for a CT angio as initially ordered. I reviewed the results which did not show evidence of active bleeding but does show diverticulosis.	Jeremy Mark Gonda, MD	07/10 2111
.	I have placed a call out to GI to discuss patient's symptoms and workup.	Jeremy Mark Gonda, MD	07/10 2112
.	I spoke with the covering gastroenterologist Dr. Takata who recommends patient be admitted to observation with serial hemoglobins, NPO at midnight and will plan on colonoscopy if he continues to bleed in the morning.	Jeremy Mark Gonda, MD	07/10 2114
.	I spoke with the patient and his spouse who agree to the plan. I reached out to the hospitalist who agrees to the admission.	Jeremy Mark Gonda, MD	07/10 2115

Dictations

None

ED Diagnoses

Diagnosis	Description	Comment	Associated Orders
Final diagnoses			
Lower GI bleed	Lower GI bleed	--	--
Diverticulosis	Diverticulosis	--	--

ED Disposition

ED Disposition	Condition	Comment
Admit	--	Level of Care: Acute [13] Supervision: No Infection Control: No

Results

Procedure	Component	Value	Ref Range	Date/Time
12 Lead ECG [480135510]				Collected: 07/10/26 2009
Order Status: Completed				Updated: 07/12/26 1337
	Ventricular Rate	90	BPM	
	Atrial Rate	90	BPM	
	P-R Interval	150	ms	
	QRS Duration	96	ms	

Results (continued)

Procedure	Component	Value	Ref Range	Date/Time
	Q-T Interval	354	ms	
	QTC Calculation (Bezjet)	433	ms	
	Calculated P Axis	33	degrees	
	Calculated R Axis	-52	degrees	
	Calculated T Axis	31	degrees	
	Diagnosis	--		
		Normal sinus rhythm		
		Left anterior fascicular block		
		Abnormal ECG		
		No previous ECGs available		
		Confirmed by Jack, Meg (29) on 7/12/2026 1:36:51 PM		
POC Fingerstick Glucose & Heelstick [480182301] (Normal)				Collected: 07/11/26 1304
Order Status: Completed		Specimen: Blood - Capillary		Updated: 07/11/26 1309
	Glucose (Bedside), Whole Blood	92	70 - 110	
Creatinine (I-STAT) [480138008] (Normal)				Collected: 07/10/26 1959
Order Status: Completed		Specimen: Blood		Updated: 07/10/26 2042
	Creatinine - POC	1.1	0.6 - 1.3 mg/dL	
Creatinine, IStat Request [480135208]				
Order Status: Completed		Specimen: Blood from Venous		

Imaging Results

CT Abdomen Pelvis W IV Cont (Final result)

Result time 07/10/26 21:06:15

Procedure changed from CTA Abdomen Pelvis W IV Cont

Final result by Jonathan D Hester, MD (07/10/26 21:06:15)**Impression:**

1. No definite acute abnormality in the abdomen or pelvis. No evidence of GI hemorrhage on significantly limited single contrast phase exam. Consider CT angiogram if clinical concerns persist.
2. Diverticulosis without diverticulitis.
3. Small hiatal hernia.
4. Diffuse hepatic steatosis.

Electronically Signed by: Jonathan Hester 7/10/2026 9:06 PM

Narrative:

EXAMINATION: CT ABDOMEN PELVIS W IV CONT

INDICATION: 64 years old, Male. GI bleed, lower

COMPARISON: None

TECHNIQUE: Multiple axial helical sections from the level of the hemidiaphragms to the lesser trochanters with intravenous contrast. All CT scans at this facility use dose modulation, iterative reconstruction, and/or size-based dosing when appropriate to reduce radiation dose to as low as reasonably achievable. Contrast was administered per approved written protocol.

Imaging Results (continued)

The patient has had 0 other CT scans within the last 12 months.

CONTRAST: 94mL ISOVUE 370.

FINDINGS:

Lung bases: Visualized lung bases are clear.

Heart: Visualized heart unremarkable.

Liver: Diffuse hepatic steatosis.

Gallbladder: Unremarkable.

Bile ducts: Unremarkable.

Pancreas: Normal.

Spleen: Unremarkable.

Adrenal glands: Normal.

Kidneys: Unremarkable.

Vasculature: Unremarkable.

Lymph nodes: No pathologically enlarged abdominal or pelvic lymph nodes according to size criteria.

Bowel: Small hiatal hernia. Diverticulosis without diverticulitis. No bowel obstruction. No definite changes of gastrointestinal hemorrhage on limited single contrast phase exam.

Intraperitoneal space: No free air or free fluid.

Bladder: Unremarkable.

Reproductive: Unremarkable.

Regional skeleton: No worrisome bone lesion.

Other: Unremarkable.

ECG Results**12 Lead ECG (Edited Result - FINAL)**

Collection Time	Result Time	Ventricular Rate	Atrial Rate	P-R Interval	QRS Duration	Q-T Interval	QTC Calculation (Beze t)	Calculated P Axis	Calculated R Axis	Calculated T Axis	Diagnosis
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07/10/26 20:09:02	07/12/26 13:36:55	90	90	150	96	354	433	33	-52	31	Normal sinus rhythm Left anterior fascicular block Abnormal ECG No previous ECGs available Confirmed by Jack, Meg (29) on 7/12/2026 1:36:51 PM
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Edited Result - FINAL

Code Onset/Outcome

Date and Time	Time Code Team Page d	Code Location/ Unit	Event Type	Witnessed	Conscious at Onset	Pulse Present at Onset	Respirations Present at Onset	Ventilated Pre-Arrest	Telemetry/ Cardiac Monitor	Cardiac Rhythm	Ectopy	Ectopy Frequency	Type of Ventilation	User
07/12/26 0940	--	--	--	--	--	--	--	--	--	NSR	--	--	--	HM
07/12/26 0420	--	--	--	--	--	--	--	--	--	NSR per tele tech	--	--	--	CM

Home Medications

	Taking?	Last Dose	Start Date	End Date	Provider
 psyllium (METAMUCIL) 58.6 %		--	07/02/26	--	Yala Reddy,

	Taking?	Last Dose	Start Date	End Date	Provider
powder Take by mouth once daily.					MD
 tadalafil (CIALIS) 20 mg tablet		--	04/14/25	--	Historical Orders

Medication Administration from 07/10/2026 1853 to 07/10/2026 2236

Date/Time	Order	Dose	Route	Action	Action by	Comments
07/10/2026 2050 PDT	iopamidol (ISOVUE-370) 76 % injection 95 mL	95 mL	Intravenous	Given	Chris Ramirez	--

ED Prescriptions

None

ED Medication Orders (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/11/26 1200	07/11/26 1113	albuterol (PROVENTIL) nebulizer solution 3 mL Once (RT) Route: Nebulization Ordered Dose: 3 mL	Discontinued	VOTT, SUBHA
07/11/26 1200	07/11/26 1113	lactated ringers infusion Continuous Route: Intravenous Ordered Dose: 100 mL/hr	Discontinued	VOTT, SUBHA
07/11/26 1200	07/11/26 1120	pantoprazole (PROTONIX) IV 40 mg Once Route: Intravenous push Ordered Dose: 40 mg	Last MAR action: Given	TAKATA, JAY A
07/11/26 1113	07/11/26 1113	naloxone (NARCAN) injection 0.1 mg every 2 minutes as needed Route: Intravenous Ordered Dose: 0.1 mg	Discontinued	VOTT, SUBHA
07/11/26 1113	07/11/26 1113	ondansetron (ZOFTRAN) injection 4 mg Once as needed Route: Intravenous Ordered Dose: 4 mg	Discontinued	VOTT, SUBHA
07/11/26 1113	07/11/26 1113	meperidine (DEMEROL) injection 12.5 mg Once as needed Route: Intravenous Ordered Dose: 12.5 mg	Discontinued	VOTT, SUBHA
07/11/26 1113	07/11/26 1113	hydralazine (APRESOLINE) 20 mg/mL injection 5 mg Every 15 minutes as needed Route: Intravenous Ordered Dose: 5 mg	Discontinued	VOTT, SUBHA
07/11/26 1113	07/11/26 1113	naloxone (NARCAN) injection	Discontinued	VOTT, SUBHA

Start	Ordered	Status	Ordering Provider
		0.04 mg Once as needed Route: Intravenous Ordered Dose: 0.04 mg	
07/11/26 1113	07/11/26 1113	fentanyl citrate (SUBLIMAZE) 50 mcg/mL 25- mcg Every 5 minutes as needed Route: Intravenous Ordered Dose: 25 mcg	VOTT, SUBHA
07/11/26 1100	07/11/26 1012	lactated ringers infusion Continuous Route: Intravenous Ordered Dose: 100 mL/hr	TAKATA, JAY A
07/11/26 1012	07/11/26 1012	lidocaine HCL (PF) (XYLOCAINE-MPF) 1 % injection solution 0.1 mL Once as needed Route: Infiltration Ordered Dose: 0.1 mL	VOTT, SUBHA
07/11/26 0900	07/10/26 2132	docosate sodium (COLAGE) capsule 100 mg Daily Route: Oral Ordered Dose: 100 mg	TAKATA, JAY A
07/11/26 0715	07/11/26 0707	magnesium citrate (CITROMA) solution 1 Bottle Once Route: Oral Ordered Dose: 1 Bottle	TAKATA, JAY A
07/10/26 2145	07/10/26 2132	sodium chloride 0.9 % (NS) infusion Continuous Route: Intravenous Ordered Dose: 50 mL/hr	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	acetaminophen (TYLENOL) tablet 650 mg Every 6 hours as needed Route: Oral Ordered Dose: 650 mg	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	sodium chloride 0.9 % infusion As needed Route: Intravenous Ordered Dose: 30 mL/hr	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	polyethylene glycol 3350 (MIRALAX) packet 17 gram Daily as needed Route: Oral Ordered Dose: 17 gram	TAKATA, JAY A

Start	Ordered		Status	Ordering Provider
07/10/26 2130	07/10/26 2132	senna (SENNAKOT) 8.6 mg 2-tablet Daily PRN Route: Oral Ordered Dose: 2 tablet	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	melatonin tablet 6 mg At bedtime as needed Route: Oral Ordered Dose: 6 mg	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	ondansetron ODT-dispersable tablet 4 mg Every 8 hours as needed Route: Oral Ordered Dose: 4 mg Placed in "Or" Linked Group	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	ondansetron (ZOFTRAN)-injection 4 mg Every 8 hours as needed Route: Intravenous Ordered Dose: 4 mg Placed in "Or" Linked Group	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	promethazine (PHENERGAN)-tablet 12.5 mg Every 6 hours as needed Route: Oral Ordered Dose: 12.5 mg	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	prochlorperazine-(COMPAZINE) tablet 5 mg Every 6 hours as needed Route: Oral Ordered Dose: 5 mg Placed in "Or" Linked Group	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	prochlorperazine-(COMPAZINE)-injection 5 mg Every 6 hours as needed Route: Intravenous Ordered Dose: 5 mg Placed in "Or" Linked Group	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	diphenhydramine-(BENADRYL)-capsule 25 mg 3 times daily prn Route: Oral Ordered Dose: 25 mg Placed in "Or" Linked Group	Discontinued	TAKATA, JAY A
07/10/26 2130	07/10/26 2132	diphenhydramine-(BENADRYL) 50 mg/mL-injection 25 mg 3 times daily prn Route: Intravenous Ordered	Discontinued	TAKATA, JAY A

Start	Ordered		Status	Ordering Provider
		Dose: 25 mg		
		Placed in "Or" Linked Group		
07/10/26 2130	07/10/26 2132	naloxone (NARCAN) injection- 0.1 mg every 2 minutes as needed Route: Intravenous Ordered Dose: 0.1 mg	Discontinued	TAKATA, JAY A
07/10/26 2050	07/10/26 2050	iopamidol (ISOVUE-370) 76 % injection 95 mL Once in imaging Route: Intravenous Ordered Dose: 95 mL	Last MAR action: Given	GONDA, JEREMY MARK

Code, Iso, Restraint (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/10/26 2131	07/10/26 2132	Code Status - Full Code Continuous	Acknowledged	MONTI, RYAN

ED Imaging Orders (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/10/26 1953	07/10/26 1952	CTA Abdomen Pelvis W IV Cont 1 time imaging	Canceled	GONDA, JEREMY MARK
07/10/26 1953	07/10/26 1952	CT Abdomen Pelvis W IV Cont 1 time imaging	Final result	GONDA, JEREMY MARK

ED Micro, Lab, POCT (From admission, onward)

Start	Ordered		Status	Ordering Provider												
07/12/26 0400	07/11/26 1620	CBC With Plt Count And Auto Diff AM Draw - Lab	Final result	CAYABYAB, VIVIAN C												
07/11/26 1613	07/11/26 1612	Troponin I, High Sensitivity Reflex Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C												
07/11/26 1435	07/11/26 1434	Magnesium, Plasma or Serum Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C												
07/11/26 1434	07/11/26 1433	CBC With Plt Count And Auto Diff Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C												
07/11/26 1434	07/11/26 1433	Comprehensive Metabolic Panel Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C												
07/11/26 1308	07/11/26 1309	Hematocrit Once - Routine - Lab Comments: stat	Final result	LIKAJ, LORENA												
07/11/26 0000	07/10/26 2132	Hemoglobin and Hematocrit Every 6 hours - Lab	Canceled	TAKATA, JAY A												
		<table><tr><th>Order ID</th><th>Start</th><th>Status</th><th>Ordering Provider</th></tr><tr><td>480140468</td><td>07/11/26 0000</td><td>Final result</td><td>TAKATA, JAY A</td></tr><tr><td>48015234</td><td>07/11/26</td><td>Final result</td><td>TAKATA,</td></tr></table>	Order ID	Start	Status	Ordering Provider	480140468	07/11/26 0000	Final result	TAKATA, JAY A	48015234	07/11/26	Final result	TAKATA,		
Order ID	Start	Status	Ordering Provider													
480140468	07/11/26 0000	Final result	TAKATA, JAY A													
48015234	07/11/26	Final result	TAKATA,													

Start	Ordered	Status	Ordering Provider
	6 0600 JAY A		
	4801 07/11 Final 6433 /26 result 9 1200 TAK ATA, JAY A		
	4801 07/11 Canc 8484 /26 eled 8 1800 TAK ATA, JAY A		
07/10/26 1953	07/10/26 1952 Creatinine, IStat Request STAT Collect/Routine Perform - Lab	Completed	MONTI, RYAN
07/10/26 1930	07/10/26 1929 CBC With Plt Count And Auto Diff STAT Collect/Perform - Lab	Final result	GONDA, JEREMY MARK
07/10/26 1930	07/10/26 1929 Comprehensive Metabolic Panel STAT Collect/Perform - Lab	Final result	GONDA, JEREMY MARK
07/10/26 1930	07/10/26 1929 Prothrombin Time/International Normalized Ratio STAT Collect/Perform - Lab	Final result	GONDA, JEREMY MARK

ED All Orders (From admission, onward)

Start	Ordered	Status	Ordering Provider
07/12/26 0400	07/11/26 1620 CBC With Plt Count And Auto Diff AM Draw - Lab	Final result	CAYABYAB, VIVIAN C
07/11/26 1613	07/11/26 1612 Troponin I, High Sensitivity Reflex Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C
07/11/26 1601	07/11/26 1600 Telemetry Monitoring Telemetry Box Continuous X 48 hours Comments:	Completed	CAYABYAB, VIVIAN C
07/11/26 1435	07/11/26 1434 Magnesium, Plasma or Serum Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C
07/11/26 1434	07/11/26 1433 CBC With Plt Count And Auto Diff Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C
07/11/26 1434	07/11/26 1433 Comprehensive Metabolic Panel Once - Routine - Lab	Final result	CAYABYAB, VIVIAN C
07/11/26 1417	07/11/26 1416 Orthostatic Vitals Daily Daily	Canceled	CAYABYAB, VIVIAN C
07/11/26 1310	07/11/26 1304 POC Fingerstick Glucose & Heelstick PROCEDURE ONCE Comments: Temp: 36.3 °C (97.3 °F)Temp Source: OralAllergies on File: Patient has no known allergies.	Final result	CAYABYAB, VIVIAN C
07/11/26 1308	07/11/26 1309 Hematocrit Once - Routine - Lab Comments: stat	Final result	LIKAJ, LORENA
07/11/26 1152	07/11/26 1151 Full Liquid Diet Continuous	Canceled	TAKATA, JAY A

Start	Ordered		Status	Ordering Provider
07/11/26 1114	07/11/26 1113	Notify Provider -- Hemodynamic Parameters Once	Canceled	VOTT, SUBHA
07/11/26 1114	07/11/26 1113	Communication Order MD to Nursing -- Discharge from PACU -- Stage 1 Continuous Comments: Discharge from PACU - Stage 1 when patient meets criteria.	Canceled	VOTT, SUBHA
07/11/26 1114	07/11/26 1113	Communication Order MD to Nursing -- May transfer from PACU when discharge criteria are met Once Comments: May transfer from PACU when discharge criteria are met.	Canceled	VOTT, SUBHA
07/11/26 1114	07/11/26 1113	Vital Signs Every 4 hours Comments: Per protocol, post procedure.	Canceled	VOTT, SUBHA
07/11/26 1114	07/11/26 1113	Oxygen Therapy -- Nursing Continuous Comments: Maintain SpO2 levels between 90% and 95%. Titrate as tolerated the liter flow or FiO2 to maintain the prescribed SpO2 levels.	Canceled	VOTT, SUBHA
07/11/26 1114	07/11/26 1113	Pulse Oximetry -- Nursing -- Continuous Continuous Comments: Maintain SpO2 levels between 90% and 95%. Titrate as tolerated the liter flow or FiO2 to maintain the prescribed SpO2 levels.	Canceled	VOTT, SUBHA
07/11/26 1114	07/11/26 1113	Cardiac Monitor -- Adult Patients Only Continuous Comments: Adult Patients Only	Canceled	VOTT, SUBHA
07/11/26 1114	07/11/26 1113	Intake and Output Continuous Comments: Per Unit Standard	Canceled	VOTT, SUBHA
07/11/26 1042	07/11/26 1042	Surgical Pathology and/or Cytology Exam RELEASE UPON ORDERING	In process	TAKATA, JAY A
07/11/26 1013	07/11/26 1012	Notify Provider -- Hemodynamic Parameters Continuous	Canceled	TAKATA, JAY A
07/11/26 1013	07/11/26 1012	Vital signs Continuous Comments: Per unit protocol	Canceled	TAKATA, JAY A
07/11/26 1013	07/11/26 1012	Oxygen Therapy -- Nursing Continuous Comments: Maintain SpO2 levels between 90% and 95%. Titrate as tolerated the liter flow or FiO2 to maintain the prescribed SpO2 levels.	Canceled	TAKATA, JAY A
07/11/26 1013	07/11/26 1012	Pulse Oximetry -- Nursing -- Continuous Continuous	Canceled	TAKATA, JAY A

Start	Ordered	Status	Ordering Provider																				
		Comments: Maintain SpO2 levels between 90% and 95%. Titrate as tolerated the liter flow or FiO2 to maintain the prescribed SpO2 levels.																					
07/11/26 1013	07/11/26 1012	Peripheral IV Continuous Comments: 20 gauge or larger. May use lidocaine 0.5% for start.	TAKATA, JAY A																				
07/11/26 1013	07/11/26 1012	POC Fingerstick Glucose & Heelstick Request STAT Collect/Routine Perform - Lab Comments: Pre-Op blood glucose evaluation, PRN as patient condition necessitates. Diabetic patients	VOTT, SUBHA																				
07/11/26 0638	07/11/26 0637	Sigmoidoscopy Once	Final result TAKATA, JAY A																				
07/11/26 0000	07/10/26 2132	Hemoglobin and Hematocrit Every 6 hours - Lab	Canceled TAKATA, JAY A																				
		<table> <tr> <th>Order ID</th><th>Start</th><th>Status</th><th>Ordering Provider</th></tr> <tr> <td>4801 4046 8</td><td>07/11 /26 0000</td><td>Final result</td><td>TAKATA, JAY A</td></tr> <tr> <td>4801 5234 6</td><td>07/11 /26 0600</td><td>Final result</td><td>TAKATA, JAY A</td></tr> <tr> <td>4801 6433 9</td><td>07/11 /26 1200</td><td>Final result</td><td>TAKATA, JAY A</td></tr> <tr> <td>4801 8484 8</td><td>07/11 /26 1800</td><td>Canceled</td><td>TAKATA, JAY A</td></tr> </table>	Order ID	Start	Status	Ordering Provider	4801 4046 8	07/11 /26 0000	Final result	TAKATA, JAY A	4801 5234 6	07/11 /26 0600	Final result	TAKATA, JAY A	4801 6433 9	07/11 /26 1200	Final result	TAKATA, JAY A	4801 8484 8	07/11 /26 1800	Canceled	TAKATA, JAY A	
Order ID	Start	Status	Ordering Provider																				
4801 4046 8	07/11 /26 0000	Final result	TAKATA, JAY A																				
4801 5234 6	07/11 /26 0600	Final result	TAKATA, JAY A																				
4801 6433 9	07/11 /26 1200	Final result	TAKATA, JAY A																				
4801 8484 8	07/11 /26 1800	Canceled	TAKATA, JAY A																				
07/10/26 2133	07/10/26 2132	Up Without Assistance Continuous	Canceled TAKATA, JAY A																				
07/10/26 2132	07/10/26 2132	No VTE Prophylaxis Because... Once	Completed MONTI, RYAN																				
07/10/26 2132	07/10/26 2132	NPO except for medications, with ice chips and sips Continuous	Canceled TAKATA, JAY A																				
07/10/26 2131	07/10/26 2132	Sequential Compression Device Bilateral; Knee Length Continuous	Canceled TAKATA, JAY A																				
07/10/26 2131	07/10/26 2132	Communication Order MD to Nursing - Order Blood Culture x2 for fever greater than 100.4 and no blood cultures in last 24 hours. Continuous	Canceled TAKATA, JAY A																				

Start	Ordered	Status	Ordering Provider
		Comments: Order Blood Culture x2 for fever greater than 100.4 and no blood cultures in last 24 hours. Obtain from two different sites.	
07/10/26 2131	07/10/26 2132	Notify Provider - No Bowel Movement Continuous Comments: If patient has not had a bowel movement for 2 consecutive days, with or without laxatives.	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	Nasal Sanitizing Protocol Continuous Comments: Order Nasal Sanitizer Protocol [NUR771] if indicated by history of infection or colonization with MRSA.	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	Vital Signs - Continuous - Per Unit Protocol Continuous Comments: Per Unit Protocol.	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	Peripheral IV Continuous	MONTI, RYAN
07/10/26 2131	07/10/26 2132	Authority is delegated to the clinical RD to modify/initiate nutrition related orders as needed per protocol Continuous	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	Notify Provider - If pain remains uncontrolled Continuous Comments: If patient on oral with IV breakthrough; notify provider if thirty minutes after repeat IV escalation dose, pain remains uncontrolled.	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	Communication Order - Provider to Nursing: Use the numeric rating scale (NRS) when patients can reliably report pain and Critical Care Pain Observation Tool (CPOT) in ED/ICU for patients who cannot reliably report pain. Continuous	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	Communication Order - Provider to Nursing: Pulse Oximetry Per Policy. Continuous Comments: Per policy for patients on opiate pain management.	TAKATA, JAY A
07/10/26 2131	07/10/26 2132	Communication Order - Provider to Nursing: If patient is difficult to arouse and/or has respiratory depression (respiratory rate of 10 or less), stop any opiates and	TAKATA, JAY A

Start	Ordered		Status	Ordering Provider
		call a Rapid Response. Continuous		
07/10/26 2043	07/10/26 1959	Creatinine (I-STAT) PROCEDURE ONCE Comments: Temp: 36.8 °C (98.3 °F)Temp Source: TympanicAllergies on File: Patient has no known allergies.	Final result	GONDA, JEREMY MARK
07/10/26 1953	07/10/26 1952	CTA Abdomen Pelvis W IV Cont 1 time imaging	Canceled	GONDA, JEREMY MARK
07/10/26 1953	07/10/26 1952	CT Abdomen Pelvis W IV Cont 1 time imaging	Final result	GONDA, JEREMY MARK
07/10/26 1930	07/10/26 1929	CBC With Plt Count And Auto Diff STAT Collect/Perform - Lab	Final result	GONDA, JEREMY MARK
07/10/26 1930	07/10/26 1929	Comprehensive Metabolic Panel STAT Collect/Perform - Lab	Final result	GONDA, JEREMY MARK
07/10/26 1930	07/10/26 1929	Prothrombin Time/International Normalized Ratio STAT Collect/Perform - Lab	Final result	GONDA, JEREMY MARK

Discharge Orders (From admission, onward)

None

Allergies (Reviewed on: 07/11/26)

No Known Allergies

Tetanus Up To Date

None

Medical History

No past medical history on file.

Surgical History

Procedure	Laterality	Date	Comment	Source
INGUINAL HERNIA REPAIR	Right	2022	Elevated BP in PACU	

Social History

Tobacco History

Smoking Status

Never

Smokeless Tobacco Use

Never

Alcohol History

Alcohol Use Status

Yes

Drinks/Week

**7 Standard drinks or
equivalent per week**

Amount

7.0 standard drinks of alcohol/wk

Drug Use

Drug Use Status

Not Currently

Frequency

0 times/week

Sexual Activity

Sexually Active
Not Asked

Other Factors

Not Asked

Social Documentation

No social documentation on file.

Family Medical History

None

ED LDA Documentation (7/9/2026 00:00 to 7/10/2026 22:36)

7/10/2026	Event	Details	User
19:47	Peripheral IV 07/10/26 Right Forearm 20 Gauge Placed	Final Assessment Date/Final Assessment Time: 07/12/26 1026 Placement/First Assessed Date/Placement/First Assessed Time: 07/10/26 1947 Placed by External Staff: Clinic Inserted by: Jenny RN Laterality: Right Location: Forearm Size (Gauge): 20 Gauge	Jennifer Clark, RN
19:47:51	Peripheral IV 07/10/26 Right Forearm 20 Gauge Assessment	Line Status: WDL; Flushed; Normal Saline Locked Site Assessment: WDL Dressing Change Due Date: 07/17/26 Dressing Change: 07/10/26 Dressing Type: Transparent Dressing Status: WDL Dressing Intervention: New Dressing	Jennifer Clark, RN

ED Events

Date/Time	Event	User	Comments
07/10/26 1849	Emergency encounter created	MAYNARD, ALEJANDRA	--
07/10/26 1853	Patient arrived in ED	MAYNARD, ALEJANDRA	--
07/10/26 1923	Triage Started	DAVIS, BROOKE	--
07/10/26 1925	Triage Completed	DAVIS, BROOKE	--
07/10/26 1929	Assign Nurse	GRIFFIN, ISABELLA	Isabella Griffin, RN assigned as Registered Nurse
07/10/26 1939	Patient roomed in ED	CLARK, JENNIFER	To room 025
07/10/26 1940	Assign Attending	GONDA, JEREMY	Jeremy Mark Gonda, MD assigned as Attending
07/10/26 1940	Assign Physician	GONDA, JEREMY	--
07/10/26 1940	Registration Completed	BOSTON, ZACHARY	--
07/10/26 2130	Remove Attending	MONTI, RYAN	Jeremy Mark Gonda, MD removed as Attending
07/10/26 2130	Assign Attending	MONTI, RYAN	Ryan Monti, MD assigned as Attending
07/10/26 2131	Bed Requested	GONDA, JEREMY	Requested: General Medicine
07/10/26 2232	Assign Nurse	JIMENEZ-TORRES, MARISOL	Marisol Jimenez-Torres, RN assigned as Registered Nurse
07/10/26 2236	ED Tracking End	BROSKEY, LACEY	--
07/10/26 2236	Patient admitted	BROSKEY, LACEY	--
07/10/26 2301	ED Boarder Patient	GONDA, JEREMY	--

Follow-up Information

Follow-up Information (continued)

Follow up With	Specialties	Details	Why	Contact Info
Jeffrey Woodrow Sanders, MD	FAMILY MEDICINE	Follow up		1946 Old Hot Springs Rd Carson City NV 89706-0674 775-882-1324

Discharge Instructions

None

Discharge References/Attachments**Lower Gastrointestinal (GI) Bleeding (Stable) (English)****AVS Reports**

Date/Time	Report	Action	User
7/12/2026 10:30 AM	Patient Discharge Instructions	Printed	Hayley McPhee, RN
7/12/2026 10:28 AM	Patient Discharge Instructions	Printed	Hayley McPhee, RN

Communication Routing History

Recipient	Method	Sent by	Date Sent
MARIA Fax: 702-800-8168	Fax	Stephany Loza Franco	7/13/2026

ED Charges

Description	Code	Date	Service Prov	Modifiers	Qty	Status
HB MEDICAL/ONCOLOGY OBSERVATION	1000146	07/12/2026			11	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	07/12/2026	Takata, Jay A		1	Filed
PROGRESS NOTE	IDAILY	07/12/2026	Takata, Jay A		1	Filed
HB CBC WITH PLT COUNT AND AUTO DIFF	3100386	07/12/2026	Cayabyab, Vivian C		1	Filed
HB VENIPUNCTURE (NON ARUP)	3000002	07/12/2026	Cayabyab, Vivian C		1	Filed
HB MEDICAL/ONCOLOGY OBSERVATION	1000146	07/11/2026			10	Filed
HB MED/ONC TELE OBSERVATION	1000145	07/11/2026			2	Filed
HB MED/ONC TELE OBSERVATION	1000145	07/11/2026			10	Filed
HB TROPONIN I	3101808	07/11/2026	Cayabyab, Vivian C		1	Filed
HB WILLOW REV 636	5040098	07/11/2026	Takata, Jay A		4	Filed
HB COMPREHENSIVE METABOLIC PANEL	3100145	07/11/2026	Cayabyab, Vivian C		1	Filed
HB MAGNESIUM, PLASMA OR SERUM	3100073	07/11/2026	Cayabyab, Vivian C		1	Filed
HB CBC WITH PLT COUNT AND AUTO DIFF	3100386	07/11/2026	Cayabyab, Vivian C		1	Filed
HB HEMATOOCRIT	3100390	07/11/2026	Likaj, Lorena		1	Filed
HB GLUCOSE BLOOD, REAGENT STRIP	3009669	07/11/2026	Cayabyab, Vivian C		1	Filed
HB HEMOGLOBIN	3100391	07/11/2026	Takata, Jay A		1	Filed
HB HEMATOOCRIT	3100390	07/11/2026	Takata, Jay A		1	Filed
HB WILLOW REV 636	5040098	07/11/2026	Takata, Jay A		1	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	07/11/2026	Takata, Jay A		1	Filed

ED Charges (continued)

Description	Code	Date	Service Prov	Modifiers	Qty	Status
CONSULT	ICONCONSULT	07/11/2026	Takata, Jay A		1	Filed
HB HEMOGLOBIN	3100391	07/11/2026	Monti, Ryan		1	Filed
HB HEMATOCRIT	3100390	07/11/2026	Monti, Ryan		1	Filed
HB VENIPUNCTURE (NON ARUP)	3000002	07/11/2026	Monti, Ryan		1	Filed
HB EMERGENCY DEPT VISIT 5	6000010	07/10/2026	Gonda, Jeremy Mark	25	1	Filed
HB HEMOGLOBIN	3100391	07/11/2026	Monti, Ryan		1	Filed
HB HEMATOCRIT	3100390	07/11/2026	Monti, Ryan		1	Filed
HB MED/ONC TELE OBSERVATION	1000145	07/10/2026			1	Filed
HB OBSERVATION	1000046	07/10/2026			1	Filed
HB WILLOW REV 258	5040080	07/10/2026	Monti, Ryan		1	Filed
HB WILLOW REV 255	5040040	07/10/2026	Gonda, Jeremy Mark	OPR	95	Filed
HB ISTAT CREATININE 82565	3009033	07/10/2026	Gonda, Jeremy Mark		1	Filed
HB COMPREHENSIVE METABOLIC PANEL	3100145	07/10/2026	Gonda, Jeremy Mark		1	Filed
HB PT, INTERNATIONAL NORMALIZED RATIO	3100360	07/10/2026	Gonda, Jeremy Mark		1	Filed
HB CBC WITH PLT COUNT AND AUTO DIFF	3100386	07/10/2026	Gonda, Jeremy Mark		1	Filed
HB VENIPUNCTURE (NON ARUP)	3000002	07/10/2026	Gonda, Jeremy Mark		1	Filed